



Physical Health Care Escalation for Patients in a Mental Health Setting SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All ward areas - exclusion Bidi Wungen Kaat (Transitional Care Unit [TCU])	Medical, Nursing, Allied Health

Purpose

This standard operational procedure (SOP) provides structured guidance on the escalation of physical health care needs for people receiving care in a dedicated mental health inpatient setting at Royal Perth Hospital (RPH) and Bentley Health Service (BHS). Actions are to be aligned with professional responsibilities, the Mental Health Act (MHA) 2014 and the corresponding [Chief Psychiatrists Standards for Clinical Care: Physical Health Care of Mental Health Consumers](#) and National Safety Quality Health Standards (NSQHS).

Links within document

- [RPBG decision making flowchart](#)

Definitions

Term	Definition
MDT (Multidisciplinary Team)	- A team of health care staff usually comprised of a variety of disciplines. In the mental health context, this typically includes psychiatry, clinical psychology, nursing, occupational therapy, social work, pharmacy, peer support workers, Aboriginal Health Liaison Officers (AHLO), welfare officers and other relevant groups who collaborate in the assessment, planning and delivery of care. - Provides specialised input to ensure that the person accessing care is provided with appropriate assessment, evidence-based interventions, assertive care, and discharge planning.
SAFE / BeSAFE Team (Safety All Hours for Everyone)	- An MDT available 24 hours / 7 days per week to support treating teams in managing people either experiencing or at risk of clinical deterioration, in line with the RPBG RPBG SAFE / BeSAFE SOP. - SAFE operates at RPH and BeSAFE operates at BHS.
AORC (Adult Observation and Response Chart)	Chart used to record physiological observations for people aged 16 and over.

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ADDs (Adult Deterioration Detection System)	The system used to score physiological observations on the AORC chart in persons aged 16 and over, that are added together to identify clinical deterioration
MET Medical Emergency Team)	An MDT comprising senior clinicians who will respond to medical emergency calls ("Code Blue"/MET calls) which are triggered by staff either due to meeting pre-determined thresholds in ADDS scores and/or due to staff concern regarding clinical deterioration.
Support people	Includes carers, family members, and other people identified by a person accessing care as a personal support person.

Principles

- Physical and mental health care must both be delivered in a culturally safe and person-centred approach.
- Where available, support people are to be recognised and included in care planning and escalation decisions, which will consider the wishes of the person receiving care and/or aligned with substitute decision maker status.
- All clinical decisions must be documented with rationale and safety considerations, including for transfers between settings.
- If physical health care needs necessitate care escalation due to deterioration or requirements exceed the capability or safety of the mental health ward to provide physical health care, then the decision process to determine appropriate care location will be made by following the [RPBG decision making flowchart](#) in conjunction with the [RPBG Recognising and Responding to Acute Deterioration Policy](#) and [RPBG Clinical Handover Policy](#).

Transfer of care to inpatient general health setting

- The decision to transfer care to the general health setting to manage physical health care provision requires decision making and consultation as per the [RPBG decision making flowchart](#)
- Transfers from the mental health setting to the general health setting must occur as per the [RPBG General Health Admission, Transfer and Discharge SOP](#) and require the following:
 - Consultant to Consultant verbal and written handover including all relevant information as per the [RPBG Clinical Handover Policy](#), including, but not limited to:
 - Current risk assessment and management plan including all details relevant to the consumer's mental health condition and its management in a general health setting
 - Treatment Support and Discharge Plan (TSDP)
 - What Matters Most
 - Current medication regime and efficacy

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Transfer of care to inpatient general health setting cont'd

- Support people
- Safety planning and mental health management strategies
- Mental health follow-up plan in general health setting
- Escalation pathways for mental health deterioration, inclusive of out of hours
- Verbal and written nursing handover including all relevant information as per the [RPBG Clinical Handover Policy](#), including, but not limited to:
 - Current diagnosis and symptoms
 - Triggers and warning signs of deterioration of mental health
 - Coping strategies, inclusive of medication and psychosocial strategies e.g. sensory modulation, music.
 - Support people
 - Safety planning and mental health management strategies
 - Escalation pathways for mental health deterioration, inclusive of out of hours.
- Involvement of the person and their support people (critical care transfers should not be delayed pending this involvement but efforts are to be made when the situation is less critical to involve the person and support people)
- Notification and handover to the RPH Consultation and Liaison (CL) Team by the treating mental health team considering current risk and TSDP and MHA 2014 legal status
- Within RPH, the CL team must review the patient within 24 hours of admission to a general health ward (including over weekends and public holidays), must regularly review the patient and ensure the MDT is kept informed (minimum daily) of changes in status, plans and risk mitigations relevant to the consumer's mental health condition.

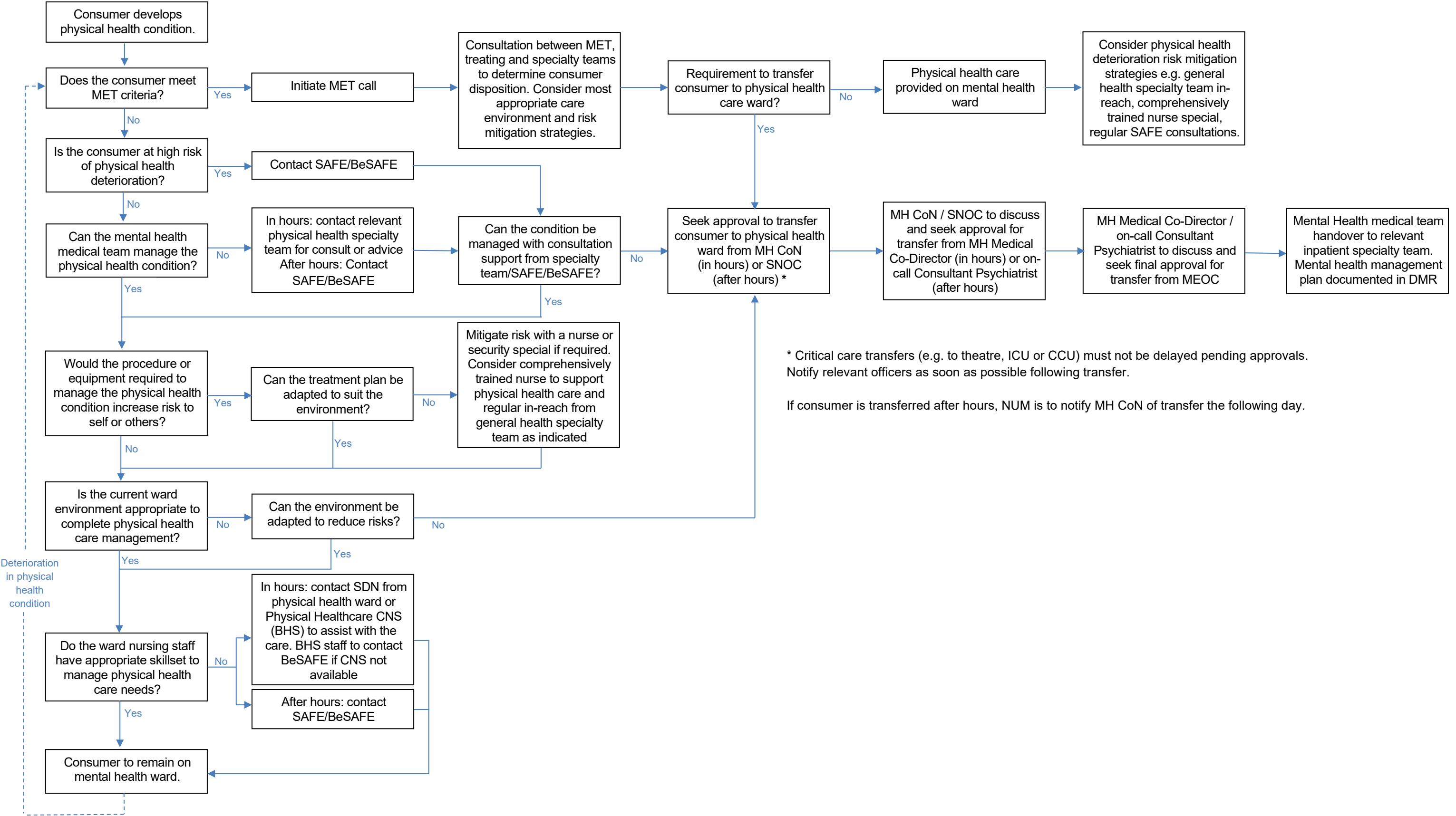
Documentation

- Documentation must follow the [EMHS Clinical Documentation Policy](#)
- All escalation decisions, rationale, interventions, and communication must be documented comprehensively in the healthcare record and the TSDP.

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R PBG decision making flowchart



Compliance monitoring

Site Executive Directors and RPBG Mental Health Co-Directors are responsible for ensuring awareness, implementation and monitoring of compliance with this procedure, including identifying risks and training needs.

Compliance monitoring and review will occur through Clinical Incident Management (CIM) reporting and investigation processes.

Facilitator

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Authors / acknowledgements

We acknowledge the following previous site endorsed work and/or contributors used to compile this document.

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Aboriginal Health Impact Statement and Declaration

TBA

Related policies, practice standards, clinical guidelines

[RPBG Policy Hub](#)

- [Recognising and Responding to Acute Deterioration Policy](#)
- [Clinical Handover Policy](#)
- [Special and Supportive Observations Policy](#)
- [Escalation to Consultants Policy](#)
- [General Health Admission, Transfer and Discharge SOP](#)
- [Medical Emergency and Cardiopulmonary Arrest Management Clinical Practice Standard](#)
- [Vital Signs for Adults Nursing Practice Standard](#)

[EMHS Policy Hub](#)

- [Clinical Documentation Policy](#)

[Department of Health Policy Hub](#)

- [MP0171/22 Recognising and Responding to Acute Deterioration](#)
- [MP 0155/21 Statewide Standardised Clinical Documentation](#)

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Other

- [CP-Clinical-Care-Standards-Physical-Health-Care-of-Mental-Health-Consumers.pdf](#)
- [Medication Safety in Mental Health](#)
- [Physical Healthcare for People living with MH issues.](#)
- [Chief Psychiatrists Standards for Clinical Care: Physical Health Care of Mental Health Consumers](#)
- [Royal Australian and New Zealand College of Psychiatrists expert consensus statement for the treatment, management and monitoring of the physical health of people with an enduring psychotic illness](#)
- [Clinical Guidelines for the Physical Health Care of Mental Health Consumers](#)
- [Mental Health Act 2014](#)

Legislative requirements, the evidence and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy HUB.

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 5: Comprehensive Care; Standard 6: Communicating for Safety; Standard 8: Recognising and Responding to Acute Deterioration

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